



Non-Neoplastic Intestinal Disease

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 8 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.**EASY**

Which two entities make up inflammatory bowel disease?

- A. Celiac disease and Crohn disease
- B. Ulcerative colitis and ischemic colitis
- C. Crohn disease and diverticulitis
- D. Crohn disease and ulcerative colitis
- E. Ulcerative colitis and IBS

2.**EASY**

Hirschsprung disease is also known as:

- A. Intestinal malrotation
- B. Meckel diverticulum
- C. Toxic megacolon
- D. Congenital pyloric stenosis
- E. Congenital aganglionic megacolon

3.**EASY**

Which IBD characteristically shows transmural inflammation?

- A. Both equally
- B. Ulcerative colitis
- C. Neither
- D. Ischemic colitis
- E. Crohn disease

4.**EASY**

Hirschsprung disease occurs in approximately what proportion of live births?

- A. 1 in 500
- B. 1 in 2000
- C. 1 in 5000
- D. 1 in 20,000
- E. 1 in 100,000

5.**MEDIUM****CASE**

A 24-year-old has perianal fistulae and iron-deficiency anemia from malabsorption. Colonoscopy shows patchy, skip-pattern ulceration.

Which finding would you expect on biopsy?

- A. Continuous involvement from the rectum
- B. Signet ring cells
- C. Transmural inflammation with granulomas
- D. Crypt abscesses without granulomas
- E. Inflammation limited to the mucosa

6.

MEDIUM CASE

A patient with long-standing colitis develops colonic dilatation, fever and abdominal distension requiring urgent management.

This complication (toxic megacolon) is most associated with:

- A. Crohn disease
- B. Ulcerative colitis
- C. Hirschsprung disease
- D. Ischemic colitis
- E. Diverticulitis

7.

MEDIUM

Which extraintestinal manifestation is specifically biliary?

- A. Episcleritis
- B. Phlebitis
- C. Sclerosing cholangitis
- D. Erythema nodosum
- E. Sacroiliitis

8.

MEDIUM CASE

A resected bowel segment shows a thickened wall, marked serositis and subserosal lymphoid aggregates with fissuring ulceration extending through the full wall thickness.

Which disease is this?

- A. Crohn disease
- B. Hirschsprung disease
- C. Ischemic colitis
- D. Ulcerative colitis
- E. Diverticulitis

9.

MEDIUM

Non-necrotizing granulomas are found in approximately what proportion of Crohn disease cases?

- A. 5%
- B. 15%
- C. 35%
- D. 60%
- E. 90%

10.

MEDIUM CASE

A colectomy specimen shows continuous involvement of the mucosa with an abrupt transition to normal bowel, hyperemic mucosa, and numerous pseudopolyps.

Which pattern of disease is this?

- A. Crohn disease
- B. Ulcerative colitis
- C. Diverticulitis
- D. Hirschsprung disease
- E. Ischemic colitis

11.

MEDIUM

Which statement about smoking and IBD is correct, per the slide?

- A. Smoking carries a higher risk in ulcerative colitis than in Crohn disease
- B. Smoking carries a higher risk in Crohn disease than in ulcerative colitis
- C. Smoking has no association with either disease
- D. Smoking causes toxic megacolon
- E. Smoking is protective against Crohn disease only

12.

MEDIUM CASE

A neonate fails to pass meconium within 48 hours of birth and develops bilious vomiting and abdominal distension.

Which structures are absent in the affected segment?

- A. Meissner and Auerbach plexuses
- B. Peyer patches
- C. Brunner glands
- D. Taenia coli
- E. Crypts of Lieberkuhn

13.

MEDIUM

Which imaging finding is expected in Hirschsprung disease on contrast enema?

- A. A napkin-ring constriction in the left colon
- B. Diffuse dilatation of the entire colon
- C. A dilated affected segment with normal proximal bowel
- D. A narrow-caliber affected segment with proximal dilatation of normal bowel
- E. Multiple filling defects throughout the colon

14.

MEDIUM

Which factors are proposed in the pathogenesis of inflammatory bowel disease?

- A. Direct bacterial invasion with abscess formation only
- B. Chronic mesenteric ischemia
- C. Altered host-microbiota interaction, epithelial dysfunction, and aberrant mucosal immunity
- D. IgE-mediated type I hypersensitivity to food antigens
- E. A single autosomal dominant gene mutation

15.

HARD CASE

A patient with long-standing Crohn disease undergoes ileal resection for a stricture. Five years later he develops recurrent disease at the anastomosis.

Which statement best reflects the natural history of Crohn disease versus ulcerative colitis?

- A. Ulcerative colitis recurs after surgery more often than Crohn disease
- B. Recurrence after surgery is common in both diseases equally
- C. Recurrence indicates a diagnosis of ischemic colitis instead
- D. Neither disease recurs after surgical resection
- E. Recurrence after surgery is common in Crohn disease but not in ulcerative colitis

16.

HARD

Which statement correctly contrasts the malignant potential and extraintestinal manifestations of CD and UC?

- A. Both carry equal malignant potential; CD has more extraintestinal manifestations
- B. UC carries more malignant potential and more extraintestinal manifestations than CD
- C. CD carries more malignant potential and more extraintestinal manifestations than UC
- D. UC has more extraintestinal manifestations but less malignant potential than CD
- E. Neither disease has malignant potential

17.

HARD CASE

A biopsy from a colitis patient shows crypt abscesses, marked inflammation, and crypt distortion with regenerative atypia and mucin depletion, without granulomas or transmural involvement.

This pattern most supports:

- A. Ulcerative colitis
- B. Hirschsprung disease
- C. Diverticulitis
- D. Crohn disease
- E. Ischemic colitis

18.

HARD

Which mechanism is proposed for aganglionosis in Hirschsprung disease?

- A.** Viral infection of the enteric neurons
- B.** Ischemic injury to the submucosal plexus
- C.** Autoimmune destruction of the myenteric plexus
- D.** Mechanical obstruction by meconium
- E.** Premature arrest of neural crest cell migration, or premature death of ganglion cells

19.

HARD

CASE

A patient with IBD develops painful, poorly healing skin ulcers with violaceous, undermined borders on the shins.

Which extraintestinal manifestation is this?

- A.** Episcleritis
- B.** Pyoderma gangrenosum
- C.** Sacroiliitis
- D.** Erythema nodosum
- E.** Sclerosing cholangitis

20.

HARD

Which combination of findings is most specific for Crohn disease rather than ulcerative colitis?

- A.** Continuous mucosal involvement from the rectum
- B.** Higher risk of toxic megacolon
- C.** Marked pseudopolyp formation
- D.** Skip lesions, transmural inflammation, and fistula formation
- E.** Crypt abscesses limited to the mucosa

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	D	IBD encompasses Crohn disease (CD) and ulcerative colitis (UC).
2	E	Hirschsprung disease is congenital aganglionic megacolon.
3	E	Crohn disease inflammation is transmural; UC inflammation is limited to the mucosa.
4	C	It occurs in about 1 in 5000 live births.
5	C	Perianal fistulae, malabsorption and skip lesions point to Crohn disease: transmural inflammation, often with non-necrotizing granulomas.
6	B	Toxic megacolon is a recognised complication of ulcerative colitis, not Crohn disease.
7	C	Sclerosing cholangitis (and gallstones) are the listed biliary tract manifestations of IBD.
8	A	Marked transmural fibrosis, serositis, and fissuring ulcers are characteristic of Crohn disease.
9	C	Granulomas are seen in about 35% of Crohn disease cases.
10	B	Continuous, rectum-based involvement with an abrupt transition and pseudopolyps is classic for ulcerative colitis.
11	B	The slide lists a higher smoking risk with Crohn disease and a lower risk with ulcerative colitis.
12	A	Hirschsprung disease is defined by absence of both the Meissner (submucosal) and Auerbach (myenteric) plexuses: aganglionosis.
13	D	The aganglionic segment is of small caliber, with proximal dilatation of the normal innervated bowel proximal to it.
14	C	IBD pathogenesis is uncertain but involves a combination of altered host reaction to microbiota, epithelial dysfunction, aberrant mucosal immune responses, and altered microbiome composition.
15	E	Recurrence after surgery is common in Crohn disease; ulcerative colitis (a mucosal disease that can be cured by colectomy) does not recur after complete colectomy.
16	B	Per the slide, UC has more malignant potential and more extraintestinal manifestations than CD.
17	A	Crypt abscesses, crypt distortion, and mucosa-limited inflammation without granulomas support ulcerative colitis.
18	E	The distal aganglionic segment results from arrested neural crest cell migration from cecum to rectum, or premature ganglion cell death.
19	B	Pyoderma gangrenosum is a listed skin manifestation of IBD, alongside erythema nodosum.
20	D	Skip lesions, transmural inflammation and fistulae are the combination most specific to Crohn disease.

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Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.